The Hillingdon Hospitals
NHS Foundation Trust

Delivering Maternity Vaccinations within the Maternity Department

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Change History

Version	Author	Summary of changes
2.0	Julie Cooper Andrew Caunce, Chief Pharmacist	Guideline updated to include RSV vaccinations

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Aim/Purpose
To describe the systems and processes for safely delivering vaccinations to pregnant women and birthing people.

Introduction
Pregnant women and birthing people are advised to have the influenza, whooping cough (Pertussis) Respiratory Syncytial Virus (RSV**)**and Covid-19 vaccines during pregnancy.

Covid -19
COVID-19 is a novel coronavirus disease caused by SARS-CoV-2 virus and spreads between people, mainly when an infected person is in close contact with another person or an infected surface (World Health Organization, 2020). The UK government, at the beginning of the Covid 19 pandemic, asked pregnant women and birthing people to be treated as a high-risk group.

Pregnant women and birthing people who are either not vaccinated or not fully vaccinated against Covid-19 are at increased risk of becoming severely unwell if they acquire COVID-19. In some cases, this may result in intensive care admission and/or lead to their baby being born prematurely.

Seasonal Flu
Seasonal influenza has the potential to add substantially to the winter pressures usually faced by the NHS. Therefore, influenza vaccination is an important priority to reduce morbidity and mortality associated with influenza, and to reduce hospitalisations.

All providers are expected to have a planned programme for the delivery of the influenza vaccination with the national ambition being to vaccinate 75% of all clinical risk groups.
Seasonal flu vaccinations are recommended for clinical high-risk groups, including pregnant women and birthing people.

Research has shown that pregnant women and birthing people have a higher chance of developing complications if they get flu, commonly pneumonia, but also, sepsis, heart problems, meningitis and encephalitis. Therefore, offering the vaccine reduces the risk of complications and maternal death from flu.

Whooping Cough (Pertussis)
The National Immunisation Program for pregnant individuals asks Obstetricians and midwives to encourage pregnant women and birthing people to receive the pertussis vaccine, ideally between weeks 20 and 32 of their pregnancy (but up to term) to optimise protection for their babies from birth.

Pertussis can be severe in newborns often requiring a hospital admission due to complications such as pneumonia. As such, it is important to encourage women and birthing people to be immunised against pertussis at the optimal time during pregnancy in order to protect their babies.

Where maternity units offer pertussis vaccines in pregnancy, providers must share information about doses of vaccines given to individual women and birthing people with their GP.

Maternity units not offering the vaccines should continue to discuss its importance, making use of available resources, and signpost the woman and birthing person to her GP to receive the vaccine.

Pregnant women and birthing people who receive the pertussis vaccination produce antibodies to pertussis which are passed through the placenta to their baby. This provides protection to the new born baby up to 8 weeks of age, at which time they should receive their routine vaccination.

Respiratory Syncytial Virus (RSV)

Vaccinating women and birthing people in pregnancy enables them to produce RSV antibodies which pass across the placenta to provide the baby with passive protection during the first six months of infancy. All pregnant women and birthing people should be offered one RSV vaccine in every pregnancy, from week 28 or soon after, to ensure sufficient time to generate enough antibodies to protect the infant.

RSV vaccine should be offered to eligible individuals throughout the year.

The Guidance

Midwives and obstetricians will advise pregnant women and birthing people about influenza, pertussis, RSV and Covid-19 vaccines.

- Pertussis vaccine is given after 16 weeks, preferably after the anomaly scan because if there are any abnormalities it less likely to be attributed to the vaccine, and before 32 weeks gestation, as after 32 weeks gestation the production of antibodies is less effective.
- Influenza vaccine is given between the months of September and March. It is the inactivated flu vaccine.
- Covid-19 vaccine can be given at any stage of pregnancy. 8 weeks are required between the 1st and 2nd dose and the booster dose is given 12 weeks after the 2nd dose.

All pregnant women should be offered one RSV vaccine in every pregnancy, from week 28. This is ideally given in week 28 or soon after, to ensure sufficient time to generate enough antibodies to protect the infant

All women and birthing people will be offered the Pertussis, RSV and flu vaccinations during their pregnancy.

Delivery of the vaccinations

The vaccinations will take place in a dedicated clinical room co-located next to the antenatal clinic.

- Pertussis vaccinations will be by booked appointment only and will coincide with the 20 week anomaly scan.

- Seasonal Flu can be either by booked appointments or on a walk in basis.

- RSV vaccinations will be either booked appointments or on a walk in basis.

- 20 minutes are allocated for each vaccination. This will include the woman and birthing person giving informed consent for the vaccination and administration of the vaccination. Where an interpreter is needed, appointments may be extended.

- All appointments will be booked by the antenatal clinic administrative staff and a letter sent confirming the appointment.

Immunisation room

The room has the following equipment:

- Vaccine fridge
- Telephone
- Computer to log immunisations onto the National Immunisation Vaccination System
- Appointment book to document bookings and in addition what vaccines have been given.
- Chair for the woman/birthing person and her birth partner
- Clinical trolley, so that vaccines can be given next to the woman/birthing person (has a vaccination tray, plasters/ cotton wool balls and a sharps bin).
- Syringes for Covid-19 vaccines can be obtained from pharmacy if required.
- Resuscitation trolley is located in Labour Ward.

The room and surfaces are cleaned daily or more frequently as required.

Surfaces should also be wiped down between each woman/birthing person.

The fridge temperature (high, low and actual temperature) are recorded on the temperature monitoring sheet. If the vaccine fridge deviates from 2-8 degrees centigrade, the vaccines must be quarantined and the advice of the pharmacist sought to whether the vaccines can be given or need to be disposed of.

Documentation of any vaccine has been given to the woman/birthing persons should be in electronic health record and in the vaccination book. All vaccinations given must also be logged on the national immunisation system for vaccinations (RAVS).

Respiratory Syncytial Virus (RSV) Administration

- All vaccinators intending to administer Abrysvo as a single practitioner will have had five such administrations supervised by another practitioner who has been accredited to administer Abrysvo vaccine. This will be recorded on a local competency assessment document and held in a folder in the Immunisation suite.

- Vaccinators must seek a second check or defer vaccination in whatever circumstance that they feel a second check is indicated.

- Abrysvo is for intramuscular use only.

- The unopened vial is stable for 5 days when stored at temperatures from 8°C to 30°C. At the end of this period Abrysvo should be used or discarded. This information is used to guide healthcare professionals in case of temporary temperature excursions only.

Storage of reconstituted vaccine
Abrysvo should be used immediately after reconstitution or within 4 hours. Store the reconstituted vaccine between 15ºC and 30ºC. Do not freeze reconstituted vaccine.

Chemical and physical in-use stability has been demonstrated for 4 hours between 15°C and 30°C. From a microbiological point of view, the product should be used immediately. If not used immediately, in-use storage times and conditions prior to use are the responsibility of the user.

Preparation for single practitioner administration
Abrysvo vaccines for use at the beginning of a vaccination session will be checked and recorded by a second practitioner to verify that the intended vaccine is the correct one and that it is within its expiry date. A recording sheet will be kept in a folder in the Immunisation Suite with the daily temperature monitoring paperwork and will include the batch number and expiry of the stock for the RSV vaccination and the signature for both the vaccinator and person checking. See Appendix 2

Women and birthing people will be made aware of the single practitioner administration process and may opt to this or a second check process which will need to be undertaken during the appointment.

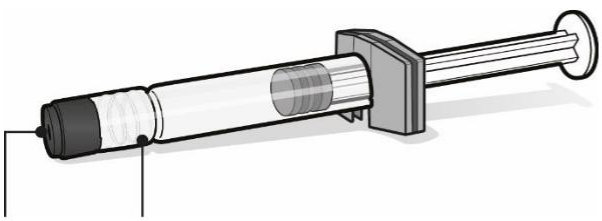
For use of vial of antigens for Abrysvo (powder), pre-filled syringe of solvent and vial adaptor

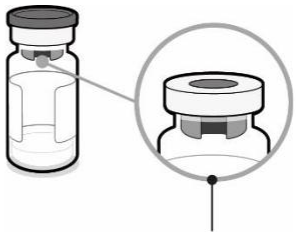
The powder must be reconstituted only with the solvent provided in the pre-filled syringe using the vial adaptor.

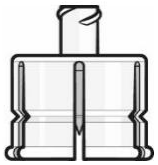
Please see instructions below.

Pre-filled syringe containing solvent for Abrysvo

Vial containing antigens
Vial adaptor for Abrysvo Powder







Syringe cap
off cap removed)

Luer lock adaptor

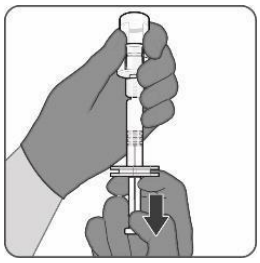
Vial stopper (with flip-

Step 1. Attach vial adaptor

- Peel off the top cover from the vial adaptor packaging and remove the flip off cap from the vial.
- While keeping the vial adaptor in its packaging, centre over the vial's stopper and connect with a straight downward push. Do not push the vial adaptor in at an angle as it may result in leaking. Remove the packaging.

Step 2. Reconstitute the powder component (antigens) to form Abrysvo

- For all syringe assembly steps, hold the syringe only by the luer lock adaptor. This will prevent the luer lock adaptor from detaching during use.
- Twist to remove the syringe cap, then twist to connect the syringe to the vial adaptor. Stop turning when you feel resistance.
- Inject the entire contents of the syringe into the vial. Hold the plunger rod down and gently swirl the vial until the powder is completely dissolved (approximately 1-2 minutes). Do not shake.



Step 3. Withdraw reconstituted vaccine

- Invert the vial completely and slowly withdraw the entire contents into the syringe to ensure a 0.5 mL dose of Abrysvo.
- Twist to disconnect the syringe from the vial adaptor.
- Attach a sterile needle suitable for intramuscular injection.

The prepared vaccine is a clear and colourless solution. Visually inspect the vaccine for large particulate matter and discolouration prior to administration. Do not use if large particulate matter or discolouration is found.

Ordering of the Vaccinations

Covid, Influenza, RSV and Pertussis vaccines are ordered via pharmacy. In England, vaccines for the routine immunisation programmes are ordered and delivered from a specialist pharmaceutical distribution company via the Department of Health's ImmForm website www.immform.dh.gov.uk The hospital pharmacy arranges this for us.

The pharmacy department in the hospital have to be informed in January about the influenza requirements for September of that year.

Immunising the woman/birthing person

Ask the woman/birthing person about which vaccine they want. It is possible to give 2 vaccines at the same time, but they must be given in different arms. If the woman/birthing person wants to have the vaccine, one at a time there must be 7 days between each vaccine.

- Ask the woman/birthing person if they have any questions about the vaccine.
- Ask the woman/birthing person about any allergies
- Ask if there have been any previous reactions to the vaccine.
- Check that the woman/birthing person has not had a fever in the last 7 days.
- If the woman/birthing person has had Covid-19 then they must wait 28 days before receiving the vaccine
- Check when the woman/birthing person last had a vaccine.
- Ask if the woman/birthing person is on anticoagulation, it is not a contraindication for the vaccine but it does mean that the vaccinator will need to apply firm pressure for a couple of minutes.
- Ask the woman/birthing person the specifically identified questions in relation to RSV vaccinations.

If there are no contraindications and the woman/birthing person has given consent then the vaccine is given intramuscularly into the deltoid muscle.

Advise the woman/birthing person of possible side effects.

Document that the vaccine has been given in the woman/birthing persons health record and in the vaccination book. It must also be logged on the national immunisation system for vaccinations (RAVS).

All vaccinations are documented on the woman/birthing persons handheld and electronic patient's record.

In order to improve the traceability of biological medicinal products, the name and the batch number of the administered product should be clearly recorded in the immunisation record book.

Data collection
Data for Flu (between September and March), RSV and Whooping Cough vaccinations will be submitted to the Immunisation commissioners on a monthly basis. See Appendix 3.

An annual immunisation report will be presented at the Maternity Governance Group (MGG) and at Divisional Board (Part A)

Associated Guidelines, Polices and Documents

- *Patient Group Direction for COVID-19 vaccine, UK Health Security Agency*
- *Patient Group Direction for Influenza, UK Health Security Agency*
- *Patient Group Direction for Pertussis, UK Health Security Agency*
- *Patient Group Direction for* Respiratory Syncytial Virus (RSV, UK Heath Security Agency
- *Patient Group Direction Policy*

Training Need Identified
All vaccinators will need to complete the national training programme training for all 4 vaccinations on https://www.e-lfh.org.uk/programmes and be authorised by name under the current Patient Group Direction for Covid-19, influenza, RSV and Whooping Cough.

The UKHSA RSV Training Slide Set for Healthcare Practitioners is below.

Respiratory syncytial virus (RSV)

The vaccinator must have completed the training for administering patient group directions.

Implementation and Monitoring
All guidelines, protocols and procedures are available on the intranet. All staff have a responsibility to check the Trust's intranet and ensure that they are referring to the most recent issue of the document. The weekly general information bulletin includes notification of publication of new/updated documents.

The administration of vaccines will be audited in accordance to the requirements stated in the Patient Group Direction Policy.

References

- all UKHSA healthcare practitioner resources relating to the RSV vaccination of pregnant women for infant protection programme can be accessed in the UKHSA RSV immunisation programme collection. These include the 'information for healthcare practitioners' document, RSV training slide set and relevant patient leaflets. A separate training side set and information for healthcare

practitioners document is also available for the RSV older adults vaccination programme

- Marketing authorisation holder's Summary of product characteristics
- Medicines and Healthcare products Regulatory Agency (MHRA): reporting adverse reactions
- The Green Book: Immunisation against infectious disease: RSV Green Book Chapter (27a)
- UKHSA Patient Group Direction (PGD): RSV vaccine
- Pfizer Abrysvo® vaccine preparation instructional video
- NHS website: Bronchiolitis
- UKHSA RSV overview: RSV: symptoms, transmission, prevention, treatment

Appendices

Appendix 1-RSV Competency Document

Appendix 2 – RSV Preparation for single practitioner administration

Appendix 3 – RSV Data Submission

Appendix 1 – RSV Competency Document

1. Purpose
To ensure that staff administering RSV vaccines using a single-person checking model are competent, confident, and compliant with national standards, local risk assessments, and governance frameworks.

2. Scope
This competency applies to all clinical staff involved in the administration of RSV vaccines within maternity services at The Hillingdon Hospitals NHS Foundation Trust, where single-person checking is implemented to improve service capacity and uptake.

3. Competency Domains

Domain	Competency Statement	Assessment Method
Legal & Ethical	Understands and applies PGD requirements and consent protocols	eLearning + Verbal Q&A
Clinical Knowledge	Demonstrates understanding of RSV, vaccine indications, contraindications, and side effects	Case-based discussion
Single Person Checking	Accurately checks patient identity, vaccine type, dose, expiry, and documentation independently	Direct observation
Emergency Preparedness	Knows how to respond to adverse reactions, including anaphylaxis and fainting	Simulation + BLS certification
Documentation	Completes digital records accurately in RAVS	Record audit
Communication	Provides clear information to patients, including opt-out and follow-up advice	Role play

4. Training Requirements

- Completion of RSV-specific immunisation training (UKHSA/NES modules)
- Familiarity with the Green Book Chapter 27a
- Completion of 5 peer supervised RSV vaccinations as a single checker

MRN	Completed by	Supervised by

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5. Declaration of Competence

I confirm that I have received the required training, understand the responsibilities of single-person checking, and am competent to administer RSV vaccines independently.

Name: _____
Signature: _____
Date: _____
Assessor Name: _____
Assessor Signature: _____
Date: _____

Appendix 2 - Preparation for single practitioner administration

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Month …….2025

Daily Second Checking of RSV Vaccinations					
Date	**Number of Vaccinations Checked**	**Batch Number**	**Expiry Date**	**Vaccinator Signature**	**Second Person Checking Signature**

Appendix 3 - RSV Data Submission

Hillingdon Monthly RSV Reports

August

Week Beginning	Date of clinic(s)	Number of available RSV appointments	Number of RSV appointments filled	Number of RSV vaccinations administered
Total				